

Gender: [REDACTED]

Birth Date: [REDACTED]

Attending: [REDACTED] (MD)

Admit Protocol: [REDACTED]

UID: F67DA6D7-9692-4400-ASC6-D7D234FC36F7
TCGA-L6-A4EU-01A-PR

Redacted



Final Results

Surgical Pathology

Final

CASE NUMBER: [REDACTED]

DIAGNOSIS:

1. "Rule out parathyroid", frozen section: Parathyroid tissue.
2. Thyroid with bilateral level VI lymph nodes, total thyroidectomy:
 - Papillary carcinoma of thyroid, tall cell variant.
 - Tumor invades muscle and perithyroidal soft tissue. Tumor extends to posterior (red) inked margin of resection and approaches the cartilage.
 - Four of thirty-three lymph nodes involved by tumor (4/33).
 - Parathyroid glands (3).
 - Thymic remnants.See Note.

NOTE:

Pathology Cancer Case Summary (Checklist)

Procedure: total thyroidectomy with bilateral level VI lymph node resection

Received: fresh

Specimen Integrity: Intact

Specimen Size

Overall, the specimen is 10.5 x 9 x 4 cm.

The right lobe is 7 x 4 x 3 cm, the isthmus is 3 x 3 x 2.4 cm, the left lobe is 5 x 2.6 x 2.3 cm.

Specimen Weight: 62.3 g

Tumor Focality: Multifocal

Dominant Tumor

Tumor Laterality: Right lobe

Tumor Size

Greatest dimension: 5 cm

Histologic Type: Papillary carcinoma

Variant, specify: Tall cell variant

Architecture: Classical (papillary)

Cytomorphology: Tall cell

Margins: Posterior resection margin (inked red) involved by tumor.

Tumor Capsule: Not encapsulated

Tumor Capsular Invasion: Cannot be assessed

100-0-3
carcinoma, papillary, tall cell variant
8344/3
Site: thyroid, Nos c73.9
hw
9/28/12

Confidential Patient Information. Unauthorized disclosure is prohibited by the Federal Privacy Act of 19

Requested By: [REDACTED] (Other)

Do not file in Medical Record

[REDACTED]		Admit Protocol: [REDACTED]
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Lymph-Vascular Invasion: Identified

Perineural Invasion: Not identified

Extrathyroidal Extension: Present

Pathologic Staging (pTNM)

Primary Tumor (pT)

pT3: Tumor more than 4 cm limited to thyroid or any tumor with minimal extrathyroid extension (eg, extension to sternothyroid muscle or perithyroidal soft tissues)

Regional Lymph Nodes (pN)

pN1a: Nodal metastases to Level VI (pretracheal, paratracheal and prelaryngeal/Delphian) lymph nodes

Specify: Number examined: 33

Number involved: 4

Distant Metastasis (pM)

Not applicable

Additional Pathologic Findings

Parathyroid gland(s): three

Thymus

Immunoperoxidase and in-situ hybridization tests performed here and used for diagnosis were developed and their performance characteristics determined by the Laboratory of Pathology. They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has also determined that such clearance or approval is not necessary. This laboratory is certified under the Clinical Laboratory Improvement Amendments of 1988 (CLIA) as qualified to perform high complexity clinical laboratory testing.

CLINICAL INFORMATION: Brief Clinical History: papillary thyroid cancer, right lobe Specimen

Taken For Protocol: 01 - Yes Allocate Order to Protocol: [REDACTED]

PROCEDURE: Pre-Operative Diagnosis: papillary thyroid cancer right lobe

Post-Operative Diagnosis: same Operative Findings: see op note

SPECIMENS SUBMITTED: 1. SERIAL 5 + 3FFR.SLIDES, Rule out parathyroid (1FS)

2. THYROIDECTOMY, TOTAL, with bilateral level 6 lymph nodes

INTRAOPERATIVE CONSULTATION: Received 1 specimen labeled with the patient's name, medical record number, date of birth, and further specified as "rule out parathyroid" are 2 tan soft tissue fragments each measuring 0.2 x 0.2 x 0.2 cm. Frozen in toto as 1FS.

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1ES diagnosis: Parathyroid tissue.

The intraoperative consultation was performed by

GROSS DESCRIPTION: Received are 2 freshly submitted specimens each in containers labeled with the patient's name, medical record number, and further specified as follows:

1. "Rule out parathyroid". The remainder of the frozen tissue is transferred from a green cassette labeled with the patient's name and 1FS to an orange cassette labeled [REDACTED].

2. "Thyroid with bilateral level 6 lymph nodes stitch right superior research and perm" is an oriented total thyroidectomy specimen with focally attached strap muscles anteriorly, focally attached shaved portion of tracheal ring on the right lobe posteriorly (1.2 x 0.8 cm), and bilaterally attached level VI lymph nodes. Overall, the specimen is 10.5 x 9 x 4 cm and 62.3 grams. There is a suture denoting the right superior lobe. The right lobe is 7 x 4 x 3 cm, the isthmus is 3 x 3 x 2.4 cm, the left lobe is 5 x 2.6 x 2.3 cm. The bilaterally attached level 6 adipose tissues containing lymph nodes is 7.5 x 2 x 1 cm. The anterior surface is inked black. The right lobe and a portion of contiguous isthmus has an indurated white-tan mass, 5 x 4.5 x 4 cm. The mass appears to abut the anterior and posterior as well as medial and lateral surfaces of the right lobe. The left lobe has a centrally positioned intact cyst (2 x 2 x 2 cm) filled with hemorrhage hazy brown watery fluid. Approximately 25% of the right indurated mass and 1 x 1 x 0.5 cm of what appears to be normal-appearing thyroid parenchyma are procured for research with equal thirds provided to Dr.

and Dr. Approximately 20% of the left lobe cyst lining is procured for Dr. [REDACTED] procured multiple FNA's from the right indurated mass and from the normal thyroid parenchyma for Dr. [REDACTED]. Gross photographs are taken. Post procurement, the posterior surface is inked red and the focally attached shave of tracheal ring on the right posterior lobe is over-inked green. The remainder of the specimen is placed into formalin and submitted to pathology for permanent processing. The procurement is performed by [REDACTED] and [REDACTED] on [REDACTED]. Also received, yet entirely procured for Dr. [REDACTED] labeled "fat for [REDACTED]" is a 1 x 1 x 0.8-cm normal-appearing adipose tissue fragment from the right cervical subcutaneous tissue. The specimen is received in Surgical Pathology and matches the above description. The specimen is sectioned and entirely submitted. Note that the cuts for procurement are made from the anterior surface.

2A - 2B: lymph nodes attached to right side (2A is 1 lymph node bisected).

2C-2F: lymph nodes and fat attached to the left lobe

2G - 2AAA: right lobe is entirely submitted from superior to inferior

2I corresponds to the area of normal-appearing tissue that was procured for research.

2N - 2AAA: firm white nodule entirely submitted. It measures 3.5 x 4 cm x 2.1 cm (post procurement).

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BBB - CCC: isthmus is sectioned and entirely submitted
 DDD - JJJ: representative sections of left lobe. No distinct
 nodules are found in the left lobe.

Representative sections of tissue that were procured from the
 lower aspect of the left lobe are placed in DDD and EEE.
 Representative sections of tissue from the area that was
 procured on the upper aspect of the left lobe are placed in
 FFF.

Gross Description dictated
 Gross Description dictated

Additional

No consultants

Final Report Signed Out

<Resident Signature>

<Sign Out Dr. Signature>

≥ 50% full cell variant per TSS.

Criteria	Yes	No
Diagnosis Discrepancy		
Primary Tumor Site Discrepancy		
HIPAA Discrepancy		
Prior Malignancy History		
Dual/Synchronous Primary Noted		
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 9/24/12	

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